

Clarify the visual symptom

Tempo, task and associated symptoms narrow the assessment before a diagnostic label is chosen.

Questions that change the approach

Tempo

Sudden onset with late presentation differs from progressive blur over months.

Task

Separate near from distance blur, central difficulty from peripheral loss.

Associated symptoms

Ask about pain, redness, double vision, trauma and previous ocular disease.

Double vision

Persistence with one eye alone suggests a monocular ocular cause; disappearance with either eye covered suggests binocular misalignment.

Connect the answer to the test

Distance acuity cannot answer every visual complaint. Add near testing for near difficulty, fields for peripheral loss, and pupil or colour comparisons when the eyes differ in visual quality. Previous surgery and medication history can clarify new blur, but first establish what the patient means by reduced vision and when it began.

Unexplained loss needs careful reassessment

Demonstrating preserved function does not establish intentional deception.

Consistency tests

What information does the performance provide?

Fogging the better eye

Useful letter reading can demonstrate capacity in the reportedly poor eye.

Repeat near and distance acuity

Look for reproducible performance rather than accepting one inconsistent result.

Field testing at different distances

A normal field widens with distance; an unchanged tunnel raises a consistency concern.

Keep the clinical interpretation safe

A complete ocular and neuro-ophthalmic examination, including the fundus, is needed to exclude an organic explanation. Functional visual loss is considered when reported or measured dysfunction lacks an adequate organic basis. Explain discrepancies neutrally and repeat assessment; an inconsistent response does not establish malingering, and a normal external appearance alone is insufficient.